

DECODED EXPLAINED

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India and Belgium Ramp Up Bilateral Defence Cooperation

India and Belgium ramp up bilateral defence cooperation

After meeting his Belgian counterpart, Prime Minister calls for ending conflicts in Ukraine and West Asia through dialogue; three government-level defence agreements signed; two sides also announce the establishment of a fast trade mechanism

Kallol Bhattacharjee
NEW DELHI

Prime Minister Narendra Modi on Thursday called for dialogue and diplomacy to end the ongoing conflicts in Ukraine and West Asia.

Mr. Modi made the remarks while welcoming Belgian Prime Minister Bart De Wever here, where the two sides ramped up bilateral defence cooperation by signing three government-level defence agreements.

Belgian sources informed that at least 10 private defence agreements will be sealed during the visit that will include production of Belgian military items such as rockets in India.

“Whether it is Ukraine or West Asia, we support all efforts aimed at an early end to conflicts and the restoration of peace. And



Boosting ties: Prime Minister Narendra Modi with Prime Minister of Belgium Bart De Wever in New Delhi. SUSHIL KUMAR VERMA

eliminating terrorism in all its forms and manifestations is our shared commitment,” Mr. Modi said in his remarks after he and Mr. De Wever discussed global and regional issues.

Paid tributes

The two leaders also paid tributes to the more than 9,000 Indian soldiers who perished in the Flanders

Fields during the First World War. A Joint Statement issued at the end of the discussions announced that the two leaders called for “safety and security of maritime routes and maintaining safe and unimpeded maritime shipping” in the context of the conflicts in West Asia and Ukraine.

The Belgian dignitary expressed support to In-

dia’s campaign against cross-border terrorism and condemned the Pahalgam terror attack.

In his remarks at an event organised by the CII (Confederation of Indian Industry), the Belgian leader described the current global situation as “turbulent” and called for free trade and end to unilateral imposition of tariffs.

The two governments signed a Letter of Intent of Defence Cooperation, an MoU between Belgian Security and Defence Industry and Society of Indian Defence Manufacturers, and an MoU between the Central Bureau of Investigation and Belgium Federal Police on cooperation in combating transnational organised crime, cyber-crime and related matters.

“Our security does not stop at our borders. That is why we need to work more closely with strong partn-

ers such as India – militarily, technologically and industrially,” said Belgian Defence Minister Theo Francken.

On this occasion, India announced that a Defence Attache will be appointed at the Indian Embassy in Brussels. The visit included defence partnerships between Belgian and Indian defence majors. “Today, India and Europe are speaking the language of trade with renewed conviction, a language of peace, of progress and of prosperity,” said Mr. De Wever.

The two sides announced the establishment of a fast trade mechanism that will deal with trade and investment and announced that bilateral trade will be doubled in the next five years. They also announced the establishment of Consular Dialogue between New Delhi and Brussels.

India and Belgium Ramp Up Bilateral Defence Cooperation

- A densely populated, industrialized sovereign country in northwestern Europe, functioning as a federal constitutional monarchy and representative democracy.
- Capital: Brussels.
- Border Nations: Bounded by four countries: France, Luxembourg, Germany, and the Netherlands.
- Hosting the key institutions of the EU, European Commission, and NATO, Belgium serves as the diplomatic and administrative hub of the Western alliance.



Forging Consensus: India at the SCO Bishkek Summit

Forging consensus

India must broaden engagement with regional multilateral groups

Prime Minister Narendra Modi's attendance at the annual meeting of Shanghai Cooperation Organisation (SCO) leaders in Bishkek, Kyrgyzstan, marking 25 years of the Eurasian grouping, had a multi-pronged agenda. This was an occasion to meet his counterparts, for the first time since the U.S.-Israel war on Iran began more than six months ago, which has inflicted economic costs on the whole grouping. The grouping includes Iran, as well as Pakistan that has emerged a mediator between Iran and the U.S., and the Bishkek declaration that condemned the U.S. and Israel's attacks on Iran and supported Iran's right to peaceful nuclear technology under the NPT, was significant. Many of the SCO members are also in the BRICS grouping who Mr. Modi will host in Delhi on September 12-13 – this includes Russian President Vladimir Putin, Chinese President Xi Jinping and Iranian President Masoud Pezeshkian. New Delhi is keen to forge consensus on a joint declaration, which has been difficult due to UAE-Iran tensions and Mr. Modi's discussions with Mr. Pezeshkian included BRICS cooperation. India's ties with the Central Asian Republics have flagged due to other pre-occupations, making Mr. Modi's visits to Tashkent and Bishkek, and meetings with regional leaders this time, an important initiative to revive talks on trade, critical minerals, tourism and other areas. As Pakistan takes over the SCO chairmanship, the Bishkek Summit allowed Mr. Modi to set out India's agenda for the coming year. His speech at the Summit focused on the importance of dismantling the ecosystem of "terrorist financing, recruitment, radicalisation, and safe havens", calling on the SCO to eschew "double standards" on terrorism. He also pressed for a strong message to countries that "use terrorism as an instrument of policy and provide safe haven and support to terrorists" – a pointed reference to India's oft-repeated concerns about Pakistan. The Bishkek declaration's inclusion of strong language on "all forms" of terrorism, including cross-border, has thus been welcomed by New Delhi. It remains to be seen how far the Pakistan presidency will take these ideas forward.

Compared to the high-octane optics and the Modi-Putin-Xi handshake at the 2025 Tianjin SCO Summit, this year's summit was relatively sombre. The U.S.'s tariffs, its war with Iran and sanctions on Russia have damaged economic security, energy security and connectivity (the Hormuz Strait blockade), and pose major challenges to the grouping. For India, after tensions with Pakistan had peaked last year after the Pahalgam attacks and the four-day conflict, the past year has been sobering, given Pakistan's diplomatic forays with the U.S., and the new security pact with Saudi Arabia and Türkiye. In the altered geopolitical framing, India must seek to broaden its engagement with all regional multilateral organisations such as the SCO.

We Need One Ministry of Energy, Not Five

We need one Ministry of Energy, not five



PRATIK
AGARWAL

Energy security today is no longer about having enough coal or enough renewable capacity. It is about ensuring the entire system works as one

INDIA IS entering a fundamentally different era of energy. For decades, our challenge was simple: Produce more energy. Coal, oil, gas, renewables and nuclear could each be planned independently because each largely operated within its own ecosystem. Today's challenge is no longer producing energy. It is optimising an increasingly complex energy system. A unit of solar power generated at noon has a different value from a unit of hydro power during the evening peak. Batteries, pumped storage and gas turbines all exist to balance renewable generation. Nuclear provides a steady baseload. Coal provides flexibility rather than simply volume. Every source now plays a different role at a different time of the day. Most importantly, every one of them depends on the same electricity grid. This changes how governments need to think about energy. India still governs the sector as though every fuel exists in isolation. Coal sits in one ministry. Oil and gas in another. Renewables in a third. Electricity in a fourth. Nuclear in a fifth. Each ministry has well-defined objectives. But each is optimising only one part of the system. Nobody is responsible for optimising the whole. That may have worked in the 20th century, but not for the 21st. Renewable projects are commissioned before transmission is ready. Gas-based plants were built without assured fuel. Coal production, imports, railway logistics and electricity demand have at times moved on different trajectories. Each ministry has largely achieved its own objectives, yet the overall system has often fallen short. The problem is not policy. It is governance.

Energy security today is no longer about having enough coal or enough renewable capacity. It is about ensuring the entire system works as one. That requires integrated planning across generation, fuel supply, transmission, storage and demand. Every investment should be evaluated based on how it improves the performance of the whole system, not one technology in isolation. Many countries have recognised this reality. The US, Brazil, Saudi Arabia and the UK have unified energy ministries responsible for looking across the energy landscape rather than individual fuels.

India should do the same. A single Ministry of Energy would not eliminate specialised expertise. Coal, oil, renewables and nuclear would continue to need dedicated departments and technical institutions. But strategy would come from one place. Infrastructure would be planned as one system. Investment decisions would be made with a single objective: Delivering the lowest-cost, most reliable and most secure energy mix for India.

As India moves towards becoming a developed economy, energy will become one of our biggest competitive advantages — or one of our biggest constraints. Our institutions need to evolve as quickly as our energy system has. Creating a unified Ministry of Energy may well be the single most important governance reform India can undertake to strengthen its energy security.

The writer is chairman, Resonia and Serentica Renewables and managing director, Sterlite Electric

We Need One Ministry of Energy, Not Five

- **The Changed Nature of the Challenge**

- For decades, India's energy challenge was simply producing more : coal, oil, gas, renewables, and nuclear could be planned independently since each largely operated within its own ecosystem.
- Today's challenge is optimising an increasingly complex system: a unit of solar at noon has different value from hydro at evening peak, and every source now plays a different role at a different time while depending on the same grid.

- **The Governance Mismatch**

- India still governs the sector as though every fuel exists in isolation, coal, oil and gas, renewables, electricity, and nuclear each sit in separate ministries, each optimising only its own part, with nobody responsible for optimising the whole.
- Consequences are visible: renewable projects get commissioned before transmission is ready, gas plants are built without assured fuel, and coal production, imports, and electricity demand move on different trajectories 'the problem is not policy, it is governance.'

- **The Case for a Single Ministry**

- Energy security today means ensuring the entire system works as one, requiring integrated planning across generation, fuel supply, transmission, storage, and demand, countries like the US, Brazil, Saudi Arabia, and the UK already have unified energy ministries.
- A single Ministry of Energy wouldn't eliminate specialised expertise : coal, oil, renewables, and nuclear would still need dedicated departments but strategy, infrastructure planning, and investment decisions would come from one place with a single objective.

- **The Stakes**

- As India moves towards a developed economy, energy will become one of its biggest competitive advantages or biggest constraints creating a unified Ministry of Energy may be the single most important governance reform to strengthen energy security.

More Than 40 Crore Indians Are Caught in a Healthcare Gap: Panel



Queue for better facilities: The threat is the loss of household income through medicines, tests and repeated treatment. (A. An)

More than 40 crore Indians are caught in a healthcare gap: panel

Hindu Shajin Peragadan

More than 40 crore Indians – over a quarter of the population – are caught in a healthcare gap – too well off to rely on government facilities, but not financially secure enough to absorb the rising cost of private care, a Parliamentary Standing Committee on Health and Family Welfare has flagged. Its latest report, ‘Affordability and Accessibility of Healthcare Facilities in Public and Private Sector’, presented to Parliament last month, makes 500 recommendations on efficacy, accessibility and regulation.

Where care is sought

The vulnerability is rooted in where Indians seek care, states the report, with more than 60% of inpatient care and 70% of outpatient care delivered by the private sector, leaving households without comprehensive financial protection exposed to private prices.

The burden extends beyond hospitalisation, with medicines accounting for nearly 30% of current health expenditure, according to the committee. More worryingly for people with diabetes, hypertension or cardiovascular disease, medicines, consultations and diagnostic tests can become a lifelong expense given that India is registering a steady increase in non-communicable disease burden.

This exposes a structural fault line in the healthcare system where the poorest have government protection, the affluent can buy private care, but the vast group in between is increasingly exposed to high costs without adequate financial protection.

For the ‘missing middle’, the threat may therefore not be a single catastrophic medical bill, but the slow erosion of household income through medicines, consultations, tests and repeated treatment.

The committee has linked this vulnerability to low public spending on health. Government health expenditure is 0.4% of GDP, against the 2.3% target under the National Health Policy, 2017. Health spending as a share of total government expenditure also fell from 0.5% in 2013-14 to 0.49% in 2022-23, below the 0.5% recorded in 2019-20.

The consequences are visible in the cost of care. Average out-of-pocket spending on childbirth is ₹15,630 in private facilities against ₹2,290 in public facilities – more than 8 times higher.

Government schemes such as Ayushman Bharat PM-JAY provide hospitalisation protection to vulnerable households, but the ‘missing middle’ often falls outside fully subsidised coverage. Private insurance, meanwhile, can leave households facing premiums, exclusions, waiting periods and co-payments.

What is needed

The committee has therefore argued that insurance alone cannot close the gap. It has called for stronger public healthcare infrastructure, nationwide implementation of the Clinical Establishments Act, mandatory quality standards and greater price transparency in private facilities.

It has also flagged retail pharmacy practices, including excessive trade margins and the non-prescription sale of antibiotics, recommending stronger audits and enforcement. The committee’s recommendations point to a problem larger than insurance coverage. The unresolved question is how to protect the millions who fall between the two.

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More Than 40 Crore Indians Are Caught in a Healthcare Gap: Panel

- **The Finding**

- More than 40 crore Indians over a quarter of the population are caught in a healthcare gap: too well off for government subsidies, but not financially secure enough for rising private care costs, per a Parliamentary Standing Committee report making 368 recommendations.

- **Where Care Is Sought and Why It's a Problem**

- Over 60% of inpatient and 70% of outpatient care is delivered by the private sector, leaving unprotected households exposed to private prices; medicines alone account for nearly 30% of current health expenditure.
- This exposes a fault line: the poorest have government protection, the affluent can buy private care, but the 'missing middle' faces the slow erosion of household income through medicines, consultations, and repeated treatment rather than one catastrophic bill.

- **The Root Cause : Low Public Spending**

- Government health expenditure is 1.43% of GDP, against the 2.5% National Health Policy target;
- The consequences show in costs: average out-of-pocket childbirth spending is ₹37,630 in private facilities versus ₹2,299 in public ones, more than 16 times higher.

- **What Is Needed**

- Ayushman Bharat-PM-JAY protects vulnerable households, but the 'missing middle' often falls outside coverage; private insurance can leave households facing premiums, exclusions, and co-payments, insurance alone cannot close the gap.
- The committee wants stronger public healthcare infrastructure, nationwide Clinical Establishments Act implementation, mandatory quality standards, price transparency, and stricter pharmacy-margin and antibiotic-sale audits.

The Gap in Manufacturing Sector GVA

The gap in manufacturing sector GVA

According to the National Accounts Statistics, the manufacturing sector's GVA is ₹38.6 lakh crore in 2023-24 at current prices; however, an 'Alternative Estimate' using data from the Annual Survey of Industries and Annual Survey of Unincorporated Sector Enterprises gives a lower figure, prompting a closer look at the official estimate

ECONOMIC NOTES

Jatinder S. Bedi
R. Nagaraj

Early this year, the National Statistical Office (NSO) released the new series of the National Accounts Statistics (NAS). It shows that the manufacturing sector's gross value added (GVA) is ₹38.6 lakh crore (₹38.6 trillion), constituting 14.7 per cent of GDP (gross domestic product) for the year 2023-24 at current prices. How reliable is the official estimate? This article investigates.

The manufacturing sector has two parts. One comprises all registered factories employing 10+20+ workers with/without power, including registered companies (defined as the organised/formal sector). The other comprises unincorporated/ informal/ household units consisting of small factories/workshops falling outside the corporate/factory sector. The Annual Survey of Industries (ASI) reports the production accounts of the factory sector, while the Annual Survey of Unincorporated Sector Enterprises (ASUSE) covers the informal sector. The combined output (GVA) of the factory and unincorporated sectors almost completely represents total manufacturing output.

The discrepancy in estimates

For 2023-24, the sum of the GVA of units covered by the ASI and ASUSE is ₹27.4 lakh crore (the Alternative Estimate or AE). The official figure, as per the NAS, is ₹38.6 lakh crore. It is higher than the AE by a whopping 40.9 per cent. Let's call this the GAP. What could account for it?

While minor variations between the two estimates is understandable due to methodological or definitional issues, such a substantial GAP between the two estimates using official data sources surely raises many questions.

What are the data sources used for estimating the official GVA? For the unincorporated sector, it is ASUSE - the same as what we have used to estimate the AE. Hence, it cannot account for the GAP noted above. Moreover, the unincorporated sector's share in total manufacturing GVA is a mere 13.9 per cent.

Hence, the reason for the GAP must lie in the estimation of organised manufacturing output. The NAS uses company balance-sheet data sourced from the Ministry of Corporate Affairs' database (MCA21), compiled using the annual statutory filings by registered companies. This practice of using the MCA data began with the previous NAS revision (base year 2014-15), partially replacing the ASI. The procedure has continued in the latest revision, with minor modifications.

A standard way to validate the GVA is to use employment data to estimate the sector's potential output by applying appropriate "technical ratios" drawn from the ASI and ASUSE datasets. The official Periodic Labour Force Survey (PLFS) for 2023-24, estimated that the manufacturing sector employed 697.5 lakh workers.

However, as per the ASI and ASUSE datasets, only 532.9 lakh workers were employed to produce the official GVA. Thus, quite possibly, the contribution of the remaining 164.6 lakh "residual workers" - 697.5 minus 532.9 - may account for the GAP in the GVA reported above.

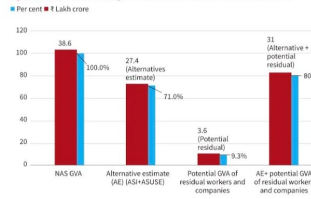


The manufacturing sector includes the organised sector comprising registered factories and companies, and the unincorporated sector comprising small factories and workshops outside the corporate sector. REPRESENTATIVE IMAGE

Conflicting figures

An alternative estimate of manufacturing sector GVA using time-tested official datasets puts the figure below the official estimate

Figure 1: Official manufacturing sector GVA and Alternative Estimates for 2023-24



The definitions of employment in the three surveys are not the same, and their data collection methods differ. However, for a validation exercise, the PLFS estimates provide a useful reference point. Official agencies also use similar methods.

Some of this GAP is contributed by the residual 177,534 MCA companies, not covered in the 78,618 "private companies" captured in the ASI data. A majority of the residual companies are likely to be non-factory private companies. The left over residual workers are likely to belong to the unincorporated sector not covered in ASUSE survey because of their small size.

Then, applying the "technical ratios" of the appropriate segments of manufacturing as derived from unit-wise

ASI and ASUSE data, the potential GVA of the residual workers is estimated to be ₹3.6 lakh crore.

Then adding the potential GVA to the AE of ₹27.4 lakh crore reported earlier, the likely potential overall manufacturing GVA could be ₹31.0 lakh crore.

This figure, however, still falls short of the official GVA estimate of ₹38.6 lakh crore by 24.5 per cent. In other words, the potential GVA of all workers employed in the manufacturing sector in companies and unincorporated sector enterprises would at best account for 80.3 per cent of the official estimate. I still leave ₹7.6 lakh crore (or 19.7%) worth of NAS manufacturing GVA "unaccounted" for or "unexplained" (Figure 1). This is the real puzzle of the new GDP figures for the manufacturing sector.

How can this GAP be reconciled with the best alternative estimate obtained using widely used data sources? The NSO documents say that, being an establishment-based survey, the ASI reportedly fails to capture value addition taking place within an enterprise, but outside of the factory premises (such as head office, marketing and distribution, or R&D activities). Is this really true? Probably not. Available evidence does not seem to support the official view (Dhokalia, Nagaraj and Pandya, *Economic and Political Weekly*, 2018).

Alternatively, if the ASI-based GVA estimate did not underestimate production, how could such a large GAP arise. Is it because of the NSO methodology of scaling up of sample estimates of active companies for the universe of companies whose size and composition are hazy and unverifiable?

The statistical issue

As per the latest NAS, for 2023-24, manufacturing GVA is ₹38.6 lakh crore at current prices. An alternative estimate, using time-tested official ASI and ASUSE data sets, finds the official figure to be higher than the alternative estimate by 40.9 per cent.

Even after accounting for the contribution of residual companies and workers, the unexplained GAP remains at 24.5 per cent. Whether the official estimate represents a "fuller description of ground reality" through the use of corporate data, or amounts to an overestimation of output, remains a matter of contention. The statistical issue can only be resolved if the MCA data and NSO's methodologies are made public for independent verification.

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THE GIST

▼ The National Accounts Statistics data shows that the manufacturing sector's GVA is ₹38.6 lakh crore for 2023-24, constituting 14.7 per cent of GDP. However, as per an Alternative Estimate, the sum of the GVA of units covered by the ASI and ASUSE is ₹27.4 lakh crore, a 40.9 per cent difference

▼ The potential GVA of 'residual workers', not captured by ASI and ASUSE data, is estimated to be ₹3.6 lakh crore. Adding this to the AE of ₹27.4 lakh crore, the potential overall manufacturing GVA could be ₹31.0 lakh crore

▼ Whether the official estimate represents a 'fuller description of ground reality' through use of corporate data, or amounts to an overestimation of output, is a matter of contention. The statistical issue can only be resolved if the NSO's methodologies are made public

The Gap in Manufacturing Sector GVA

Suppose an Indian citizen makes an investment abroad and earns a positive return on her investment. Which of the following is correct?

This question was previously asked in

**UPSC CAPF AC 2023 Official
Paper-I**

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1. Her income is part of India's GDP, but not part of India's national income.
2. Her income is part of India's national income, but not part of India's GDP.
3. Her income is part of both India's GDP and national income.
4. Her income is neither part of India's GDP, nor its national income.

The Gap in Manufacturing Sector GVA

Which of the following statements is/are correct?

1. GDP deflator captures the average price of an unchanging basket of commodities that constitutes the GDP of the country.
2. GDP deflator can be used to measure the real GDP of the economy but not the inflation rate.

Select the correct answer using the code given below.

This question was previously asked in
CDS General Knowledge 21 April
2024 Official Paper

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1. 1 only

2. 2 only

3. Both 1 and 2

4. Neither 1 nor 2

How Roman Telescope Can Help Decode Universe's Mysteries

How Roman telescope can help decode universe's mysteries

Anjali Marar

Bengaluru, September 3

NASA ON Sunday (August 30) launched a powerful, next-generation space telescope that can search for clues for some of the universe's deepest mysteries, including mysterious phenomena such as dark matter and dark energy. The telescope, launched on a SpaceX rocket, is named after the late **Nancy Grace Roman** — known as the “mother of Hubble” owing to her role in the creation of the Hubble Space Telescope, which has been operating since 1990.

What will the telescope do?

In terms of scientific capabilities, the Nancy Grace Roman Space Telescope lies between the Hubble Space Telescope and the James Webb Telescope, which was launched in 2021. It will be able to scan vast swathes of the cosmos and may provide new insights about exoplanets, dark matter and dark energy.

EXOPLANETS: These are planets located outside our solar system and are often found orbiting a star or a “remnant star” (the remains of a star that has burnt out).

So far, more than 6,000 exoplanets have been identified. The Roman Space Telescope may be able to identify many more. This search for exoplanets could eventually

help us answer that tantalising question: Are we alone in the universe?

DARK MATTER AND DARK ENERGY: Scientists estimate that ordinary matter makes up only about 5% of the universe. The rest is composed of little-understood phenomena called dark matter and dark energy.

Scientists believe that dark matter, which makes up about 27% of the universe, is an invisible glue that holds galaxies together. On the other hand, dark energy, estimated to make up around 68% of the universe, is believed to be a repulsive force that has expedited the universe's expansion since the Big Bang. This telescope will conduct the most detailed dark matter studies ever, potentially helping map how it is structured and distributed throughout the cosmos, especially during the process of early star birth.

Besides this, this telescope will help measure how gravity subtly bends the path of light across vast distances (a phenomenon known as gravitational lensing).

The Roman telescope will, therefore, provide a three-dimensional map of the distribution of galaxies and galaxy clusters across the universe.

How does it work?

The Roman Space Telescope, weighing about 8,000 kgs and about the size of a tour bus, will operate from the Lagrange Point

Who was Nancy Grace Roman?

● Born in 1925, Roman was NASA's first chief astronomer and made key contributions to space-based telescopes at a time when ground-based ones were preferred. She died in 2018.

● Roman was deeply involved in NASA's Great Observatories programme: Hubble Space Telescope, Chandra X-ray Observatory, Compton Gamma Ray Observatory, Spitzer Space Telescope. All these took astronomy to new heights



A SpaceX rocket carries the Nancy Grace Roman Space Telescope. NASA/X

2 — a vantage point between Earth and Sun located 1.2 million km from Earth.

At this point, the gravity from the Sun and Earth works together with an object's motion around the Sun. This can help the Roman telescope remain steady. Lagrange Point 2 also offers stable optical performance and an unobstructed view of the universe for imaging. The James Webb Telescope operates from this zone.

It has two major instruments on board: **WIDE FIELD INSTRUMENT:** This is a 300-megapixel infrared camera that can capture a patch of the sky about 1.5 times bigger

than the apparent size of a full Moon. The telescope can also process images 1,000 times faster than the Hubble telescope.

CORONAGRAPH: Just like humans cannot look directly into the sun but can see its rounded rim with the help of goggles/protective shields, astronomers use coronagraphs to mask the extreme bright light emitted by celestial bodies to study their surroundings. The Roman Telescope will deploy one of the most advanced coronagraphs that will block the glare from stars and help scientists to see the faint reflected light from planets around them.

For Newborns, the Answer Is Hospital Plus Home

For newborns, the answer is hospital plus home

In India, newborns are dying. Earlier, they used to die at home without medical care. Now, they are dying in hospitals. On August 24, 2026, three newborns lost their lives in an accidental fire at the government women's hospital in Amravati, Maharashtra. This was not the first such episode in government newborn care units, and it will not be the last – unless we view it not as an isolated event, but as a trend.

These are episodes of clustered newborn deaths in the government institutions (that were identified by ChatGPT)/in the format of Year/Government hospital/Newborn deaths – 2017 BRD Medical College, Gorakhpur 17; 2017 Government hospital, Ahmedabad 18; 2019 JK Lon Hospital, Kota 10; 2020 JK Lon Hospital, Kota 9; 2020 Government hospital, Shahdol 8; 2021 District Hospital, Bhandara One fire 10; 2021 Kamla Nehru Children's Hospital, Bhopal One fire 4; 2021 Government Medical College, Ambikapur 4; 2022 Medical College, Ambikapur 4; 2023 Government Medical College, Nanded 12; 2024 Maharani Laxmi Bai Medical College, Jhansi One fire 10; 2025 District Women's Hospital, Budaun 4; 2026 District Women's Hospital, Amravati One fire 3

This is not a comprehensive national registry, but only a partial list. The pattern, however, is recurring and therefore demands a systemic response.

The problems and challenges

What is going wrong? The first reason is overcrowding. India has achieved remarkable success in ensuring that women increasingly deliver in health institutions – from 39% in 2005-06 to 90% in 2023-24 (National Family Health Survey-6). But this success has created a new challenge. The absolute number of women delivering in institutions has risen from 109 lakh in 2005 to 194 lakh in 2024-25 (Ministry of Health and Family Welfare (MoHFW), Government of India). The total number of sick newborns hospitalised in public Special Newborn Care Units (SNCUs) has also risen rapidly, from 11.3 lakh in 2021-22 to 14.45 lakh in 2023-24 – a 28% increase in two years (Health Management Information System, MoHFW). Hence, newborn-care units everywhere are becoming overloaded.



Abhay Bang

Physician and the founder of Society for Education, Action and Research in Community Health (SEARCH), a community health research organisation

Newborn care in India must combine safer, decongested neonatal units with strengthened home-based support

The second reason is the changing case mix. As institutional deliveries have expanded, government hospitals have become the destination not only for normal deliveries but also for large numbers of high-risk births, including premature, low-birth-weight and sick newborns referred from peripheral facilities.

Infrastructure failures are the third reason. The Gorakhpur episode of August 2017 was attributed by a district-level inquiry to oxygen deprivation following an interruption in oxygen supply. The fires at Bhandara in January 2021, Bhopal in November 2021 and Jhansi in November 2024, as well as the more recent incident in Amravati, demonstrate the risks posed by fire and electrical hazards in neonatal units. Sick newborns need warmers, incubators and ventilators. Overcrowding can increase the electrical load on these units.

Infection and staffing problems would be the fourth reason. Overcrowding, inadequate nurse-to-baby ratios, equipment shortages and poor infection-prevention practices can turn a neonatal unit into a setting where hospital-acquired infections can spread rapidly.

The Gadchiroli model

This diagnosis leads to some obvious solutions. But there is an additional way.

India already has another solution. More than two decades ago, the Gadchiroli field trial conducted by the Society for Education, Action and Research in Community Health (SEARCH) demonstrated that trained community health workers (CHWs) could provide effective home-based neonatal care (HBNC), resulting in a 62.2% reduction in neonatal mortality in the rural area (The Lancet, 1999). The trained CHWs identified and managed neonatal infections at home, supported breastfeeding and warmth, and managed low-birth-weight and preterm babies. Subsequent data from Gadchiroli, published in the Journal of Perinatology in 2005, showed that between 1996 and 2003, 97% of low-birth-weight and preterm babies – including those with birth weights above 1,800 g and gestational ages of more than 34 weeks – were managed at home, with a very low case-fatality rate.

The principles of the Gadchiroli model have

already become a part of India's public health system (MoHFW, 2011). Around 8,00,000 Accredited Social Health Activists (ASHAs) have been trained to provide HBNC, using training modules developed from the Gadchiroli experience. Therefore, India does not have to invent a new system. ASHAs, if adequately trained, supervised and supported, can provide many components of newborn care. This could take a considerable load off SNCUs and help reduce overcrowding.

Of course, a baby with severe prematurity, respiratory distress, shock, severe sepsis, serious birth asphyxia or another danger sign needs immediate facility-based treatment. HBNC is not a substitute for neonatal intensive care. But not every newborn who needs care requires a hospital bed.

The optimum strategy

India should therefore adopt a three-part strategy. First, decongest neonatal units. Strengthen HBNC by ASHAs so that appropriate, stable newborns can safely receive care at home rather than occupying scarce SNCU beds.

Second, strengthen neonatal care units. Ensure adequate numbers of doctors and nurses, appropriate nurse-to-baby ratios, functioning equipment, reliable oxygen and electricity supplies with backup systems, along with rigorous infection-prevention measures.

Third, make neonatal units intrinsically safe. Fire detection and suppression systems, electrical and oxygen-system safety measures, emergency evacuation drills and independent safety audits should become mandatory.

The success of the Janani Suraksha Yojana (JSY) is unquestionable. Institutional delivery can help save mothers and babies. But the next phase of India's newborn-health strategy must recognise that getting mothers into hospitals is only the first step. Every mother will give birth to a baby who needs care. But not every newborn who needs care requires a hospital bed. The future of newborn care should not be hospital versus home; it should be hospital plus home.

That would reduce overcrowding and, most importantly, ensure that each newborn receives the right care, at the right place, at the right time.

For Newborns, the Answer Is Hospital Plus Home

- **The Recurring Pattern**

- In India, newborns are dying earlier at home without care, now increasingly in hospitals; on August 24, 2026, three newborns died in an accidental fire at a government women's hospital in Amravati, Maharashtra, one of a recurring pattern of clustered newborn deaths in government institutions dating back years.

- **Four Root Causes**

- Overcrowding: institutional deliveries rose from 39% (2005-06) to 90% (2023-24); sick newborns in public SNCUs rose from 11.3 lakh to 14.45 lakh in two years, a 28% increase, overloading units everywhere.
- Changing case mix and infrastructure failures: hospitals now handle many high-risk births; fires at Bhandara, Bhopal, Jhansi, and Amravati show fire/electrical hazards worsened by overcrowding, alongside infection risks from inadequate nurse-to-baby ratios.

- **The Gadchiroli Model : A Home-Based Alternative**

- The Gadchiroli field trial by SEARCH showed trained community health workers could deliver effective home-based neonatal care (HBNC), cutting neonatal mortality by 62.2% in the rural trial area (The Lancet, 1999).
- Subsequent data showed 97% of low-birth-weight and preterm babies were managed at home with very low case-fatality; the model is already part of India's system, with 8,00,000 ASHAs trained in HBNC using Gadchiroli-derived modules.

- **A Three-Part Strategy**

- First, decongest neonatal units by strengthening ASHA-led HBNC for stable newborns (not a substitute for intensive care in severe cases); second, strengthen SNCUs with adequate staffing, equipment, and backup power/oxygen.
- Third, make neonatal units intrinsically safe with mandatory fire detection, electrical safety, and independent audits, the future should be hospital plus home, not hospital versus home.

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- Janani Suraksha Yojana (JSY) is a safe motherhood intervention under the National Health Mission. It is being implemented with the objective of reducing maternal and neonatal mortality by promoting institutional delivery among poor pregnant women.
- JSY is a centrally sponsored scheme, which integrates cash assistance with delivery and post-delivery care. The Yojana has identified Accredited Social Health Activist (ASHA) as an effective link between the government and pregnant women
- The scheme focuses on poor pregnant woman with a special dispensation for states that have low institutional delivery rates, namely, the states of Uttar Pradesh, Uttarakhand, Bihar, Jharkhand, Madhya Pradesh, Chhattisgarh, Assam, Rajasthan, Orissa, and Jammu and Kashmir. While these states have been named Low Performing States (LPS), the remaining states have been named High Performing states (HPS).
- LPS : All pregnant women delivering in government health centres, such as Sub Centers (SCs)/Primary Health Centers (PHCs)/Community Health Centers (CHCs)/First Referral Units (FRUs)/general wards of district or state hospitals
- HPS : All BPL/Scheduled Caste/Scheduled Tribe (SC/ST) women delivering in a government health centre, such as SC/PHC/CHC/FRU/general wards of district or state hospital

Category	Rural area		Total	Urban area		Total
	Mother's package	ASHA's package*		Mother's package	ASHA's package**	(Amount in Rs.)
LPS	1400	600	2000	1000	400	1400
HPS	700	600	1300	600	400	1000

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Consider the following statements in relation to Janani Suraksha Yojana:

1. It is a safe motherhood intervention of the State Health Departments.
2. Its objective is to reduce maternal and neonatal mortality among poor pregnant women.
3. It aims to promote institutional delivery among poor pregnant women.
4. Its objective includes providing public health facilities to sick infants up to one year of age.

How many of the statements given above are correct?

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1. Only one

2. Only two

3. Only three

4. All four

LEFTOVER TOPICS

- FCRA AMENDMENT
- GDP CRITICISM

Thank You